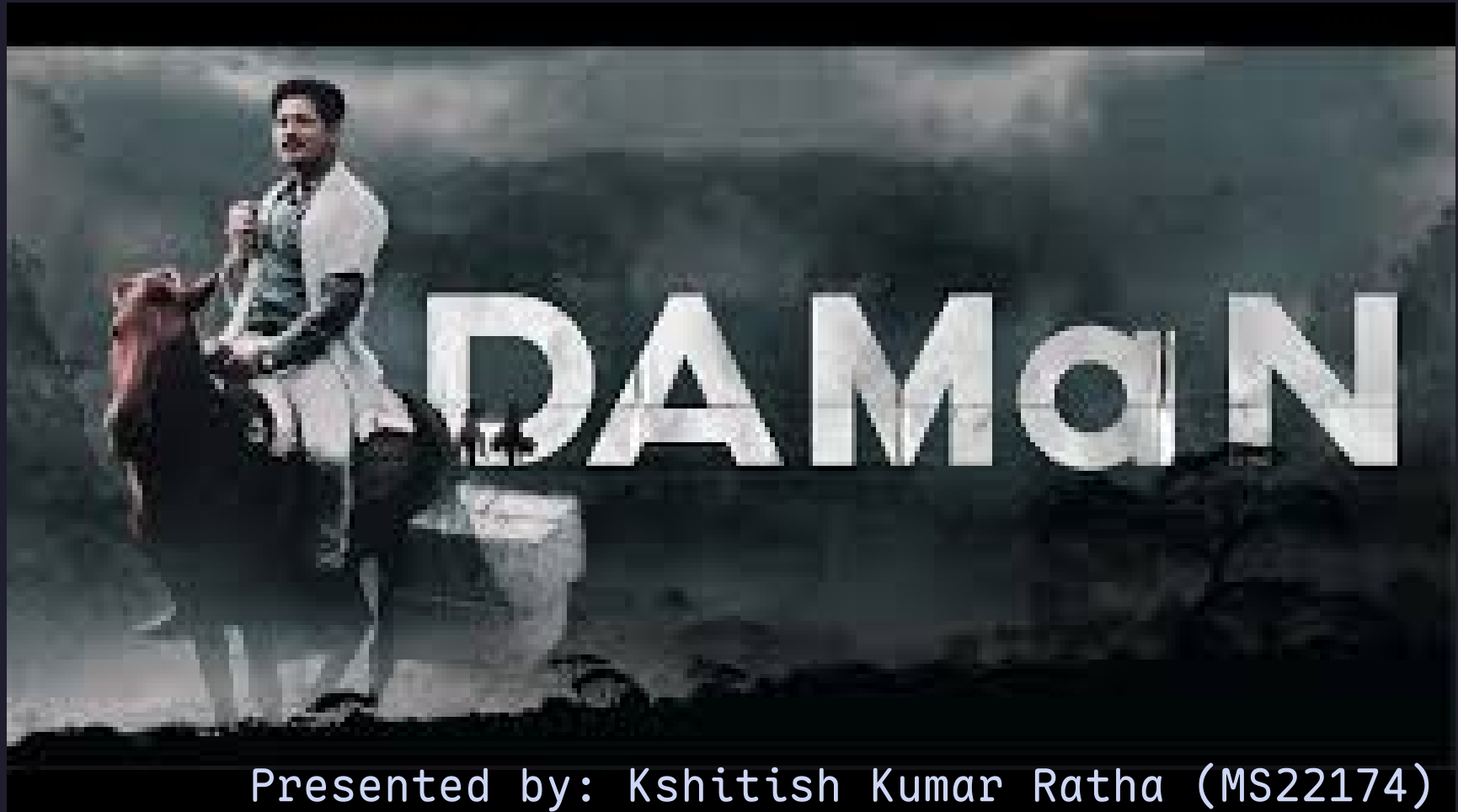




Malaria Eradication in Inaccessible Areas.

Brief Summary of the Film

- Set in 2015
- Siddharth, a young doctor, is posted to a remote tribal area of Odisha
- The region has 151 villages with minimal facilities
- Villagers suffering from malaria often rely on traditional beliefs instead of treatment
- Early interventions fail despite medicines and mosquito nets
- Over time, a structured campaign reduces malaria cases

The film is not just about one doctor. Its about the conditions under which healthcare fails.



There is nothing here.. How is someone supposed to work like this?



Figure 1: Abhijit Banerjee

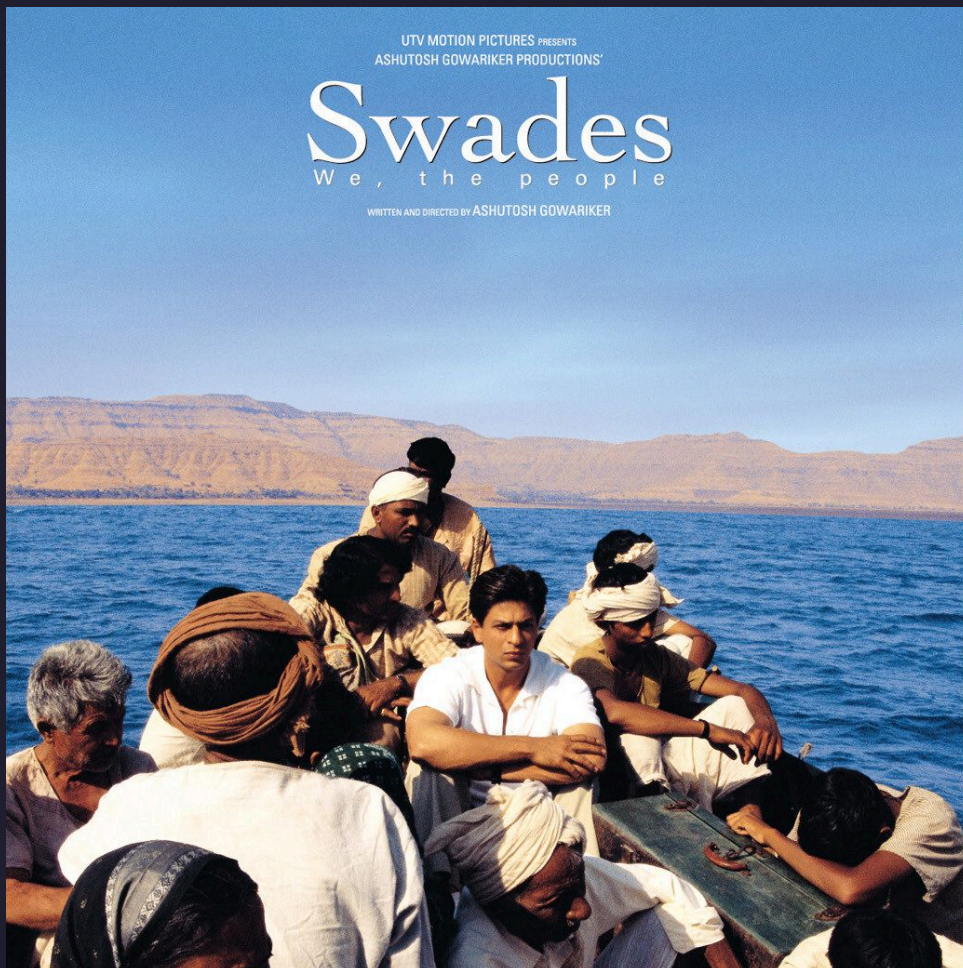


Figure 2: Swades



You think this is a spirit? This is an illness



Figure 3: Arthur Kleinmann

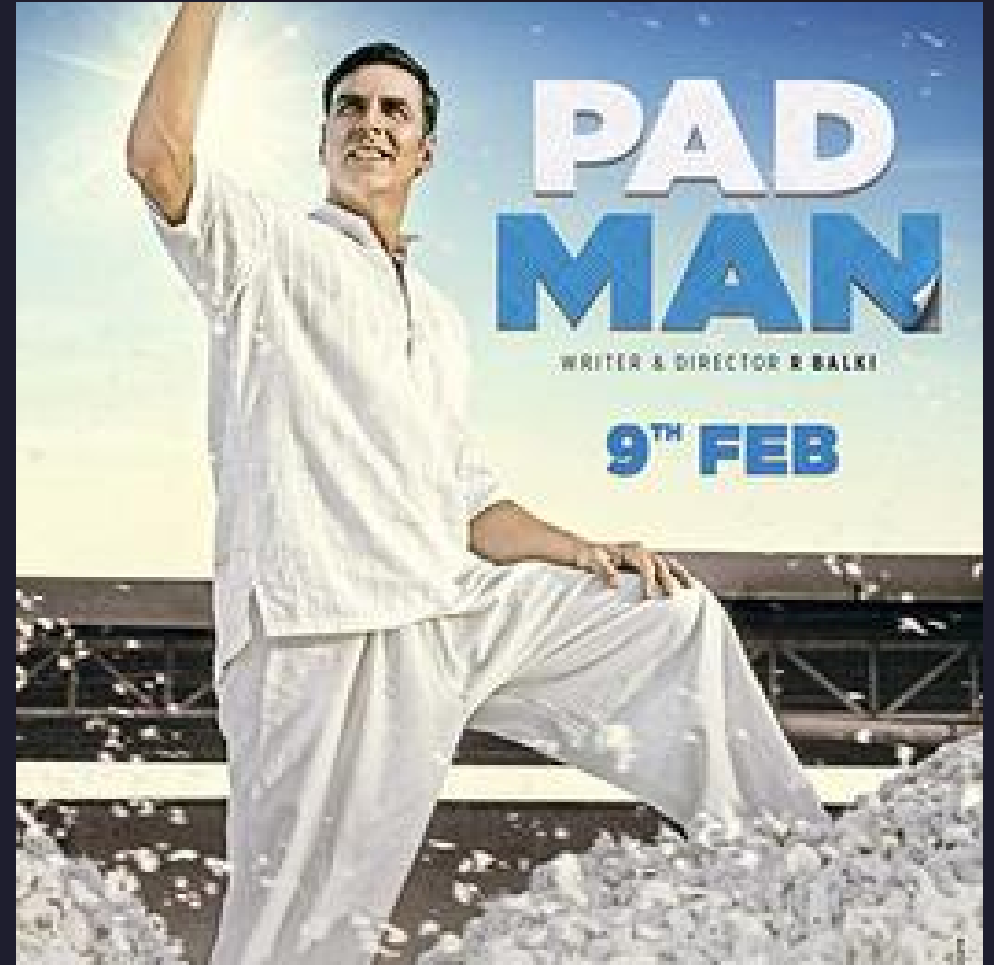
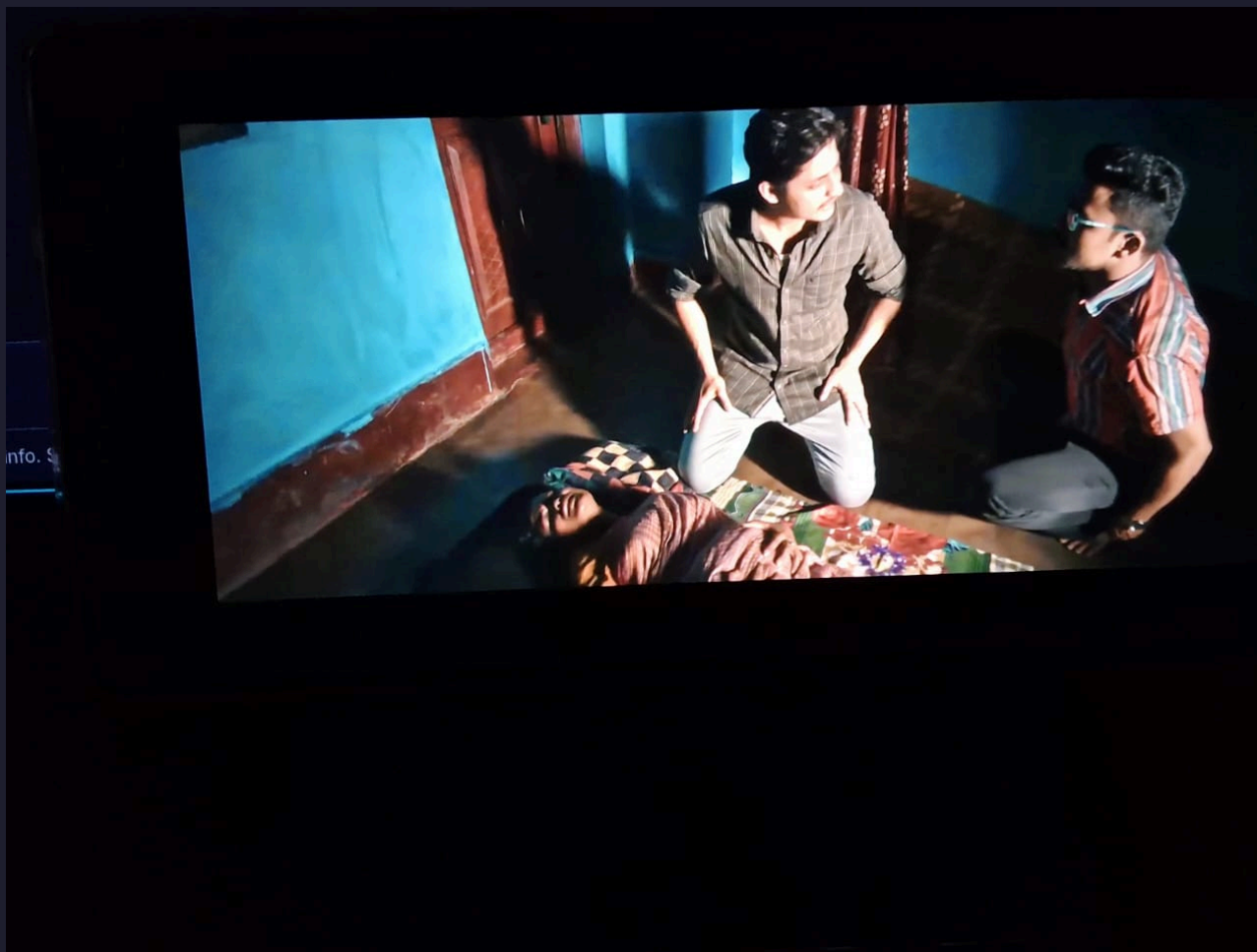


Figure 4: Pad-Man



We have to carry her... There is no other way.

Three Layers of Healthcare

We can think of healthcare in three layers:

- **Availability** – does care exist?
- **Accessibility** – can people physically reach it?
- **Usability** – can they actually make use of it?

The film shows that rural healthcare fails even when availability is partially solved.

If a patient cannot reach a hospital, then for that patient, the hospital does not exist.



They are using the nets... but not for sleeping

Why Good Interventions Still Fail

Policies often assume that once a solution is provided, people will use it in the intended way.

But people use objects according to:

- immediate needs
- habits
- environment
- local priorities

So the gap is not just between policy and people.

It is between **design** and **lived reality**.

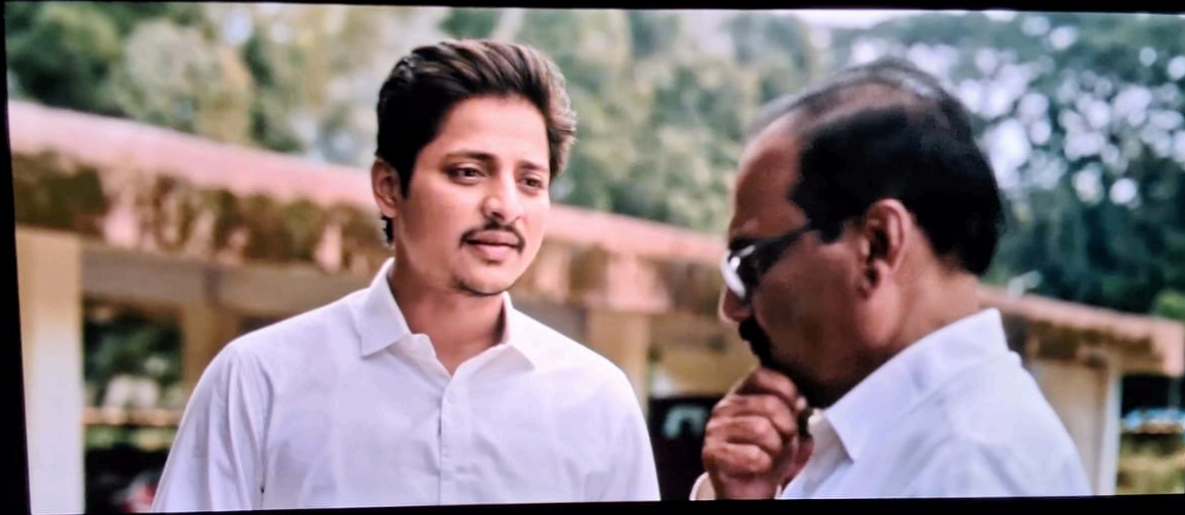
The Limits of Individual Effort

Siddharth is doing far more than one person should. He is:

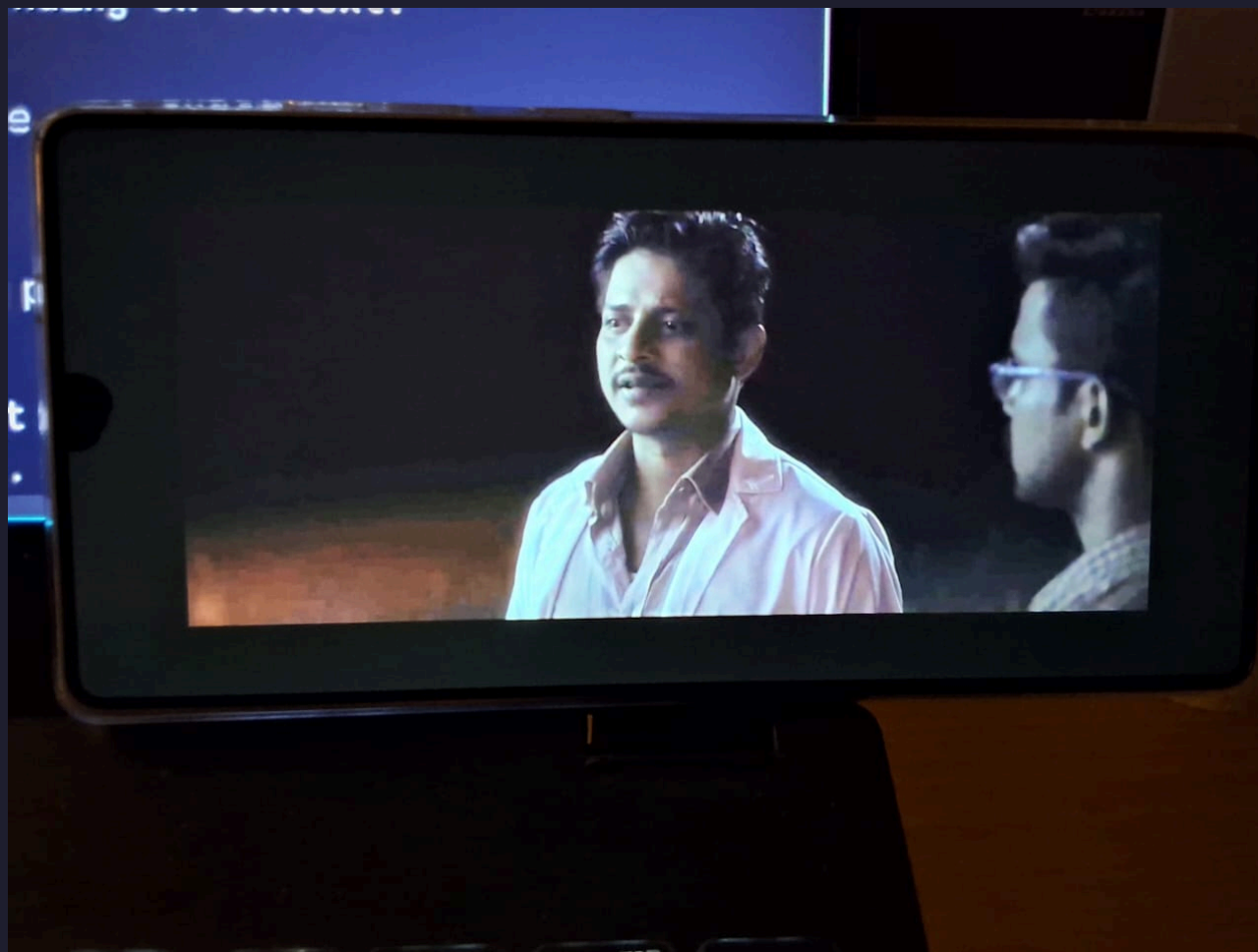
- transporting patients
- explaining medical knowledge
- distributing resources
- coordinating action across villages

They are functions that should belong to institutions, infrastructure, and networks.

He is not only treating malaria. He is compensating for the absence of a system.



There are so many villages, how can one person reach all of them?



If we don't go, no one will!



It can not depend on one person... It has to become a system.

Final Argument

Across the film, the same pattern appears again and again.

Healthcare does not fail only at the point of treatment. It fails in the conditions that make treatment possible:

- access
- communication
- policy design
- system structure

What looks like a medical problem is often a systemic one.

Conclusion

DAMaN is not just a story about a doctor fighting malaria. It is a story about how healthcare actually works. The film shows that:

- Medicine must be reachable
- Treatment must be trusted
- Policy must fit everyday life
- Care must be supported by systems, not just sacrifice

Effort can start change, but only systems can sustain it.



System Working!



People Using Malaria Nets Properly!

Cases Decreased from
40% to 4% in span of
2 years.



Figure 5: Dr. Omkar Hota carrying a
bleeding pregnant women to Malkangiri
hospital.